

Parliamentary Bill: Rehabilitation and Recovery through Cybernetic Augmentation Act 2025

A Bill for an Act to provide for lawful, safe, and effective use of cybernetic augmentation as a rehabilitative tool in health, disability, criminal justice and related settings, integrating with the Therapeutic Goods Act 1989, Privacy Act 1988, Safety, Rehabilitation and Compensation Act 1988, and associated frameworks, and for related purposes.

Short Title:

Cybernetic Rehabilitation Act 2025

PART 1-PRELIMINARY

Division 1-Short Title and Commencement

Clause 1: Short Title

This Act may be cited as the **Cybernetic Rehabilitation Act 2025**.

Clause 2: Commencement

(1) This Act commences on a day to be fixed by Proclamation. (2) Any provision of this Act not commenced within 12 months of Royal Assent shall commence on the first day of the thirteenth month following assent^[1].

Clause 3: Simplified Outline

This Act establishes the legal framework for:

- The lawful use and oversight of cybernetic augmentation for rehabilitation,
 - Defining eligibility for such interventions,
 - Establishing consent and ethical safeguards,
 - Ensuring compliance with privacy, technical, and safety standards,
 - Creating mechanisms for accreditation, reporting, and oversight,
 - Integrating new provisions into existing statutory and regulatory schemes.
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Division 2-Definitions

Clause 4: Definitions

In this Act, unless the contrary intention appears:

- **Cybernetic augmentation:** The integration of artificial components (including hardware, software, or hybrid bio-electronic systems) with the human nervous, muscular, or skeletal system, for the purpose of restoring or augmenting physical, sensory, or cognitive function.

- **Augmentation device:** Any medical, therapeutic, or assistive technology, including prosthetics, neural interfaces, limb replacements, exoskeletons, or brain-computer interfaces, intended for rehabilitation.
 - **Rehabilitation:** Programs or interventions aimed at restoring, improving, or maintaining physical, sensory, or cognitive abilities for individuals with impairment, disability, or severe injury, excluding cosmetic or enhancement purposes except where essential to functional recovery^[2].
 - **Consent:** Free, informed, and express agreement of a person, or, where lacking capacity, their legally authorised representative, in accordance with the Privacy Act 1988 and mental health legislation^[4].
 - **Personal information, health information:** As defined by the Privacy Act 1988.
 - **Commission:** The National Cybernetic Rehabilitation Commission, established by this Act.
 - **Therapeutic device:** A cybernetic augmentation device classified under the Therapeutic Goods Act 1989.
 - **Approved rehabilitation provider:** A person or entity accredited under Division 3, Part II of this Act.
 - **Serious adverse event:** Any event which leads to: death; serious injury or illness; permanent impairment; or requires hospitalisation, in connection with a cybernetic augmentation device.
 - **ARTG:** Australian Register of Therapeutic Goods.
- (Additional definitions may be listed in Schedule 1.)

Clause 5: Relationship to Other Acts

(1) This Act applies subject to the Therapeutic Goods Act 1989, the Privacy Act 1988 (including the Privacy and Other Legislation Amendment (POLA) Act 2024), the Disability Discrimination Act 1992, the Safety, Rehabilitation and Compensation Act 1988, relevant criminal justice statutes, and State and Territory mental health acts^[4]. (2) Where an inconsistency exists, the provision that affords the highest degree of individual safety, privacy, or human rights protection shall prevail, except as otherwise specified herein.

PART 2-LEGAL AUTHORITY AND SCOPE

Division 1-Object of the Act

Clause 6: Object

The object of this Act is to provide a lawful, safe and ethical framework to:

- Enable full rehabilitation through regulated cybernetic augmentation to eligible persons;
- Integrate such technologies into rehabilitation, disability, and criminal justice programs;

- Safeguard the rights, safety, and privacy of individuals receiving cybernetic interventions;
 - Promote access, equity, and best-practice rehabilitation aligned with Australia's obligations under the UN Convention on the Rights of Persons with Disabilities and national digital health strategies.
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Division 2-Scope and Jurisdiction

Clause 7: Application

(1) This Act extends to all States and Territories of Australia. (2) The Act applies to Commonwealth agencies, public and private healthcare providers, criminal justice institutions, disability service providers, and other prescribed bodies providing rehabilitation services with cybernetic augmentation.

Division 3-Lawful Rehabilitation through Cybernetic Augmentation

Clause 8: Lawful Use

(1) It is lawful for an approved rehabilitation provider to assess, provide, fit, monitor, and maintain cybernetic augmentation devices for eligible persons in accordance with the requirements of this Act, prescribed standards, and binding guidelines. (2) Cybernetic augmentation must not be provided primarily for elective enhancement or cosmetic purposes, except where necessary for therapeutic or rehabilitative function.

PART 3-ELIGIBILITY, CONSENT, AND ETHICAL SAFEGUARDS

Division 1-Eligibility Criteria

Clause 9: Eligibility Criteria and Assessment Procedures

(1) An individual is eligible for cybernetic augmentation under this Act if: (a) they have a clinical diagnosis of impairment or disability resulting in substantial functional limitation; (b) the impairment is unlikely to be resolved through less invasive or conventional means; (c) evidence-based assessment indicates that cybernetic augmentation will materially improve rehabilitation outcomes; (d) they satisfy additional criteria prescribed in the rules or Schedules; (e) for criminal justice settings, the intervention is part of a court, tribunal, or parole board-approved rehabilitative plan and subject to additional procedural safeguards.

(2) Eligibility shall be determined following a multidisciplinary assessment involving: (a) a registered medical practitioner specialising in rehabilitation medicine or surgery; (b) an accredited prosthetist or relevant allied health professional; (c) a psychosocial evaluator for capacity and informed consent; (d) a technology expert with accreditation in cybernetic rehabilitation.

(3) The assessment must be conducted in accordance with current national and international guidelines for assistive technology and rehabilitation, including standards referenced by the National Disability Insurance Scheme and Private Hospital Rehabilitation Guidelines^[6].

Division 2-Safeguards for Consent and Privacy

Clause 10: Consent

(1) Cybernetic augmentation under this Act must not be undertaken unless: (a) the eligible person provides written, informed, and voluntary consent; (b) in the case of impaired capacity, consent is provided by a person authorised under State or Territory guardianship laws or the relevant mental health act; (c) the person providing consent is given comprehensive, accessible information about the procedure, risks, alternatives, device properties, privacy protections, data processing, and rights of withdrawal or complaint; (d) where the individual is subject to involuntary mental health or criminal justice orders, additional safeguards, independent advocacy, and a statutory review mechanism (see Division 4) are provided.

(2) The consent process must comply with the Privacy Act 1988, the POLA Act 2024, the National Statement on Ethical Conduct in Human Research (current edition), and relevant Codes of Ethics for health professionals^{[7][9]}.

Clause 11: Privacy and Data Protection

(1) All personal and health information, device telemetry, and data generated by or through cybernetic augmentation devices must be: (a) collected, processed, stored, disclosed, and destroyed in accordance with the Privacy Act 1988, the POLA Act 2024, and relevant guidelines of the Office of the Australian Information Commissioner (OAIC)^[3]; (b) subject to explicit patient/proxy authorisation for any secondary use outside direct care, except as otherwise required by law; (c) protected with best-practice technical, organisational, and procedural safeguards, including encryption at rest and in transit, robust authentication, secure deletion, audit logs, and regular privacy impact assessments; (d) accessible by the individual, with rights of access, correction, erasure, review, and complaint.

(2) Serious data breaches must be notified in accordance with the Notifiable Data Breaches scheme.

Division 3-Special Safeguards in Mental Health and Criminal Justice

Clause 12: Compulsory or Involuntary Use

(1) Cybernetic augmentation may only be used in involuntary settings (including under mental health or criminal justice orders) if: (a) it is demonstrably the least restrictive, least intrusive, and most proportionate suitable intervention; (b) it is targeted at harm minimisation and rehabilitation, not punitive purposes; (c) independent legal, medical, and ethical review is available before, during, and after the intervention; (d) the individual has the right to a second opinion, to representation, and to regular statutory review as set out in Division 4.

(2) All decisions and procedures in this Division are to be consistent with the Mental Health Statement of Rights and Responsibilities, the National Statement on Ethical Conduct in Human Research, and State/Territory mental health legislation^{[11][8]}.

Division 4-Complaints, Reviews, and Representation

Clause 13: Complaints

(1) Every person subject to a cybernetic augmentation program, or their representative, may lodge a complaint with the Commission, the provider, the OAIC, or relevant bodies with respect to: (a) eligibility, process, consent, or outcome; (b) privacy or data security breaches; (c) device failure or adverse events; (d) decision-making or procedural fairness concerns.

(2) Complaints must be investigated promptly, with outcomes and remedies communicated in writing.

Clause 14: Statutory Review

(1) All decisions to impose, alter, or revoke involuntary cybernetic rehabilitation must be subject to independent, periodic review at least annually, or more frequently where requested by the individual or their representative. (2) Tribunal review rights must be maintained and accessible, subject to the Administrative Review Tribunal Act 2024 and migration/natural justice principles^[13].

PART 4-TECHNICAL, MEDICAL, AND SECURITY STANDARDS

Division 1-Registration and Approval of Devices

Clause 15: Integration with the Therapeutic Goods Act 1989

(1) All cybernetic augmentation devices to which this Act applies must be: (a) classified and registered as medical devices with the Therapeutic Goods Administration (TGA) in accordance with the Therapeutic Goods Act 1989 and the Therapeutic Goods (Medical Devices) Regulations 2002; (b) included in the ARTG at the appropriate level of classification (Class I-III) depending on risk and function^{[14][15]}; (c) subject to conformity assessment, quality management (ISO 13485), risk management (ISO 14971), and best-practice cybersecurity standards as appropriate for the level of risk and invasiveness^[17]; (d) maintained and monitored under post-market surveillance, vigilance, and recall mechanisms as required by law.

Division 2-Technical and Medical Standards

Clause 16: Cybersecurity and Data Protection Requirements

(1) All devices must meet or exceed the TGA's Essential Principles for cybersecurity and technical safety, including: (a) secure by design and by default principles across the device lifecycle^[16]; (b) encryption of all personal and sensitive data in transit and at rest; (c) unique device identification (UDI) implementation as per TGA guidelines; (d) vulnerability management, regular updates, patching, and formal incident response pathways; (e) validation, verification, and interoperability with national health and prosthesis databases (e.g., My Health Record).

Clause 17: Medical and Rehabilitation Practice Standards

(1) Cybernetic rehabilitation must be provided by, or under the clinical governance of: (a) a medical practitioner who is a Consultant Physician in Rehabilitation Medicine, or equivalent; (b) a multidisciplinary team in accordance with standards from the Australasian Faculty of

Rehabilitation Medicine, the NDIS assistive technology guidelines, and private/public hospital rehabilitation service guidance^[2].

(2) Physical infrastructure, training, equipment, and individualised program plans must be maintained as per standards for admitted and non-admitted rehabilitation programs.

Division 3-Device Supply, Procurement, and Environmental Standards

Clause 18: Procurement and Supply

(1) Procurement of devices must comply with the Environmentally Sustainable Procurement Policy, relevant Commonwealth procurement rules, and model contract clauses, including environmental sustainability plans for suppliers^[19].

PART 5-OVERSIGHT, ACCREDITATION, AND GOVERNANCE

Division 1-Establishment of the National Cybernetic Rehabilitation Commission

Clause 19: Establishment of the Commission

(1) The National Cybernetic Rehabilitation Commission is established as an independent statutory authority under this Act. (2) The Commission's functions include: (a) accreditation of providers of cybernetic rehabilitation; (b) issuing and updating codes of practice and practice directions; (c) data collection and reporting; (d) review of complaints and systemic issues; (e) advice to Ministers and Parliament on regulatory, technical, and ethical developments; (f) oversight of compliance, investigation of adverse incidents, and enforcement actions; (g) liaison with the TGA, OAIC, NDIS Quality and Safeguards Commission, clinical advisory bodies, and State/Territory health authorities.

Clause 20: Accreditation of Providers

(1) All rehabilitation providers offering cybernetic augmentation must: (a) be accredited by the Commission as Approved Rehabilitation Providers, and comply with Commission-issued operational standards and guidelines; (b) submit to regular audit and review; (c) maintain insurance, quality management, and risk management arrangements; (d) have policies and procedures for privacy, data governance, consent, complaints, and adverse event management. (2) Accreditation may be suspended or revoked for serious non-compliance, subject to procedural fairness rights.

Clause 21: Professional Standards and Training

(1) Practitioners must: (a) hold current registration and/or certification in their discipline (medical, allied health, technical); (b) complete Commission-approved training modules in cybernetic rehabilitation; (c) participate in continuing professional development and audit.

Division 2-Audit, Reporting, and Compliance Monitoring

Clause 22: Audit and Data Reporting

(1) Providers must report key data as prescribed, including: (a) numbers and demographics of persons assessed, eligible, and receiving cybernetic augmentation; (b) device types, manufacturers, supply and maintenance pathways, and UDI identifiers; (c) adverse events, device failures, recalls, or security incidents; (d) rehabilitation outcomes and functional assessments (using FIM or other AROC-accepted instruments); (e) privacy impact assessments and data breach notifications.

(2) The Commission will publish an annual report on compliance, outcomes, and systemic issues, and submit it to the Minister for tabling in Parliament.

Division 3-Enforcement and Penalties

Clause 23: Offences and Sanctions

(1) It is an offence punishable by a fine not exceeding 5,000 penalty units or imprisonment for 5 years, or both, for any person to: (a) provide cybernetic augmentation services without accreditation; (b) intentionally supply, fit, or maintain a device not approved in accordance with the Therapeutic Goods Act 1989; (c) knowingly breach consent, privacy, or data security requirements as stipulated under this Act.

(2) The Commission may require remedial action, suspend or revoke accreditations, and refer matters for prosecution or civil penalty proceedings.

PART 6-INTEGRATION WITH OTHER LEGISLATION AND SCHEMES

Division 1-NDIS and Disability Integration

Clause 24: NDIS Funding and Access

(1) Eligible persons under the NDIS Act 2013 may access cybernetic augmentation as a reasonable and necessary support, subject to assessment, planning, and cost-effectiveness criteria, and may apply NDIS funds to assessment, supply, maintenance, and training as per NDIA published guidelines^{[21][6]}.

Division 2-Therapeutic Goods Act and Digital Health Integration

Clause 25: Mandatory Compliance with the Therapeutic Goods Act 1989

(1) All registration, safety, adverse event reporting, recall, and post-market surveillance obligations under the Therapeutic Goods Act 1989 and regulations apply with equal force to cybernetic augmentation devices delivered under this Act^{[15][23]}.

Clause 26: Digital Health and Data Interoperability

(1) Devices, software, and records must meet requirements of the National Digital Health Strategy, and be interoperable with My Health Record and the Australian Unique Device Identification Database when applicable^[24].

Division 3-Privacy Act and POLA Integration

Clause 27: Privacy Act and POLA Application

(1) Privacy, data handling, and breach reporting by providers and device suppliers must comply with the Privacy Act 1988 and Privacy and Other Legislation Amendment Act 2024. (2) Providers and the Commission must undertake regular privacy impact assessments as per OAIC guidance.

Division 4-Mental Health and Criminal Justice Linkages

Clause 28: Mental Health and Justice Safeguards

(1) Where cybernetic augmentation is provided as a therapeutic measure in criminal justice or involuntary mental health settings, it must comply with: (a) State/Territory civil and forensic mental health laws; (b) Principles of the UN Convention on the Rights of Persons with Disabilities; (c) National Statement on Ethical Conduct in Human Research (where research is involved); (d) National Prisoner Health and Rehabilitation Frameworks where relevant^[10].

PART 7-STATUTORY RULES AND MISCELLANEOUS

Clause 29: Statutory Rules

(1) The Governor-General may make regulations, not inconsistent with this Act, prescribing matters required or permitted by this Act to be prescribed, or necessary or convenient for carrying out or giving effect to this Act.

Clause 30: Transitional, Savings and Application Provisions

(1) The Minister may, by legislative instrument, make rules for the purpose of prescribing transitional arrangements, savings, or application provisions to facilitate the implementation or transition to the Act.

PART 8-SCHEDULES

Schedule 1-Definitions

Schedule 1 will provide extended and technical definitions for key medical, technical, criminal justice, and administrative terms as required by this Act, and references to relevant international standards and guidelines (e.g., ISO 13485, ISO 14971, IEC 62304, NIST Cybersecurity Framework, GDPR).

Schedule 2-Model Clauses for Provider Accreditation

- Detailed procedural rules, including template contractual clauses for supply, environmental compliance, and minimum content of informed consent documentation.
- Model contract and procurement requirements for government-acquired cybernetic devices, consistent with the Commonwealth environmental sustainability policy^[19].

Schedule 3-Integration Clause Examples

- Clauses outlining explicit cross-references for Articulation with the Therapeutic Goods Act, Privacy Act/POLA, the Mental Health Act(s) of all States and Territories, and the NDIS Act, including a concordance table of clause references and their mapping into related statutory schemes^[4].

Analytical Commentary and Supplementary Provisions

Legislative Drafting Standards and Format

Structure and Format:

This bill utilises the standard Australian Commonwealth drafting structure: sequential Parts, Divisions, Schedules, and defined clause references as per OPC guidance, the Legislation Handbook, and style references^{[26][28][30][31]}. Definitions are consolidated early under a Preliminary Division, with terms cross-referenced for clarity and interpreted in bold italics as per best practice^[29].

Integration Clauses:

Cross-references to existing frameworks are explicit and embedded, avoiding ambiguity and ensuring that rights, obligations, and enforcement regimes operate seamlessly alongside other legislation. These include detailed linkages with the Therapeutic Goods Act, Privacy Act, POLA Act, the NDIS, the Safety, Rehabilitation and Compensation Act, mental health statutes, and criminal justice provisions, responding to principles established in model clauses and contemporary legislative drafting guidelines^{[4][27][19]}.

Procedural, Compliance, and Governance Architecture:

The Act establishes a dedicated Commission for oversight. Its roles span accreditation, complaint investigation, standard setting, audit, reporting, and enforcement-reflecting the administrative and governance best practices recommended in recent Commonwealth legislation and tribunal review models^[13]. The Commission is empowered to develop binding codes, audit providers, manage adverse event data, and liaise with federal, state, and territory agencies.

Consent and Ethical Safeguards:

Consent requirements are rigorous, aligned with NHMRC's National Statement on Ethical Conduct in Human Research and OAIC health privacy guidance, ensuring informed, voluntary, and revocable consent, as well as structured additional safeguards for persons under mental health or criminal justice orders^{[7][11]}.

Technical and Medical Standards:

Device approval and post-market surveillance strictly follow TGA regulations and the post-UDI reforms, including cybersecurity mandates in the TGA Essential Principles, data traceability via UDI, and international harmonisation (IMDRF, ISO, EU MDR, FDA guidance)^{[14][32][34]}. Medical and multidisciplinary standards reflect requirement for individualised plans, allied health protocols, and documented outcome measurement, as per AROC, NDIS and private/public hospital guidelines^{[35][6]}.

Privacy and Data Security:

Privacy protections are supported by mandates for PIAs, data minimisation, breach notification, robust encryption, and explicit patient rights (access, erasure, correction, withdrawal), in full alignment with the Privacy Act 1988 and the more recent POLA reforms-enabling both routine and research use, but always with explicit patient control and accountability measures^{[4][33][16]}.

Safeguards for Involuntary and Justice Settings:

Cybernetic augmentation in criminal justice or mental health contexts requires heightened review, independent advocacy, least-restrictive assessment, periodic statutory review, and integration with tribunal/administrative review architecture, drawing from recent reviews of forensic, prison, and digital rehabilitation practices in Australia and internationally^[10].

NDIS and Disability Integration:

NDIS funding, device eligibility, and assistive technology assessment processes are modelled directly on the latest NDIA guidelines-ensuring clear, fair access but requiring multidisciplinary assessment, device cost-effectiveness analysis, and transparency in procurement and supply chain arrangements^{[21][6]}.

International Harmonisation and Best Practice:

Adopts prevailing international norms for medical device regulation, cybersecurity, ethics, consent, and rehabilitation-referencing IMDRF, FDA, EU MDR/AI Act, GDPR, and NIST standards, as mandated in TGA and OAIC guidance, and reinforced by interpretive and comparative law analyses^{[17][34][9][8]}.

Supplementary Commentary on Policy Rationale

Policy Rationale and Evidence Base:

A growing body of clinical, digital health, and criminal justice research supports the use of advanced cybernetic and assistive technology as key drivers of improved rehabilitation, enhanced return to function, and social reintegration for Australians with disease, injury, or disability^[6]. The NDIS, TGA, and State public/mental health frameworks have already recognised the clinical and societal imperative for integrating new technologies into rehabilitation programs, but, to date, a coherent national statutory framework has been lacking.

The proposed Act plugs this legislative gap, enabling best-practice, rights-protective, and technologically current pathways for cybernetic rehabilitation, while addressing explicit safety and ethical risks (data breaches, medical harm, involuntary settings, consent, privacy) surfaced in recent regulatory and parliamentary reviews and international comparatives^[15].

Safeguard Rigor and Accountability:

Given the unique invasive nature and digital footprint of cybernetic devices, statutory mechanisms mandating rigorous accreditation, oversight, professional standardisation, digital health interoperability, and mandatory reporting are essential. Data and privacy compliance are set at a high bar (OAIC, Notifiable Data Breach protocols, robust encryption, right of erasure), matching national and international expectations. For involuntary settings (e.g., under mental health or justice orders), enhanced procedural rights and consent review mechanisms ensure least-restrictive and rule-of-law anchoring, consistent with Australia's human rights and disability discrimination commitments^[8].

Technical Integration and Futureproofing:

Dynamic reference to TGA, OAIC, NDIS, and environmental sustainability standards enables flexibly futureproofed compliance. The Unique Device Identifier system and modular reference to procurement sustainability mirror international moves to ensure medical device safety, traceability, preparedness for recalls, environmental soundness, and harmonised post-market responsiveness^{[19][24]}.

Consultation and Review:

The Commission's proactive role in analysis, audit, guidance, and public reporting-mirrored on the TGA, NDIS, and emerging digital regulators-builds accountability, transparency, and scope for continual improvement and public dialogue.

Conclusion

The **Cybernetic Rehabilitation Act 2025** creates a comprehensive, integrated architecture for the safe, ethical, and scientifically robust deployment of cybernetic augmentation technologies as part of Australia's rehabilitation, disability, health, and criminal justice systems. It does so through clear eligibility and consent protocols, strict integration with leading medical, privacy, and digital standards, and accountable oversight-filling the lacuna at the intersection of law, technology, and rehabilitation, and positioning Australia at the global forefront of technological and ethical best practice in this rapidly evolving sector.

END OF BILL

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